

# MVCC SLEEP TECHNOLOGIES PROJECT



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Sleep Center Management  
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PSG 225/230

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## Licensing

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## CHAPTER OVERVIEW

### Chapter 1: Introduction to the Sleep Center and Sleep Laboratory

This chapter will cover the basic components of a sleep center, its purpose, and its functions.

#### Chapter Objectives:

Define the components of the sleep center

Know the importance of sleep medicine in healthcare

Explain the different functions of a sleep center

Understand how sleep labs contribute to research and clinical practice

Describe the role of the sleep technologist in the clinic and laboratory setting

Know the standards of accreditation

Discuss the relationship between the sleep lab environment and the patient experience

#### Overview of Sleep Centers and Sleep Laboratories

Sleep centers and sleep laboratories are specialized facilities dedicated to the diagnosis, treatment, and research of sleep disorders. These centers are equipped with advanced diagnostic tools that monitor, evaluate, and manage patients experiencing a wide range of sleep-related issues. The National Heart, Lung, and Blood Institute estimates that approximately 50 to 70 million Americans have sleep disorders and that about 1 in 3 American adults do not get the recommended amount of continuous sleep ([Nat'l Heart, Lung, and Blood Inst.](#)). Worldwide, quality sleep is an issue which can lead to significant health problems if left untreated. Some of the most common conditions seen in diagnostic testing sleep facilities are different types of sleep apnea, such as (OSA), narcolepsy, circadian rhythm disorders, and sleep disordered breathing due to comorbid disorders such as ALS or Atrial Fibrillation.

So, what is the difference between a sleep center and a sleep laboratory? A sleep center refers to a facility that has a day clinic and a night laboratory. A sleep center can either be a hospital-based facility or a non-hospital-based outpatient facility ([MCD](#)). The sleep clinic portion of the sleep center is where patients are seen before and after a sleep study. Typically, the patient will be evaluated by either a physician or an advanced practitioner such as a Physician's Assistant or a Nurse Practitioner. During initial evaluation, a full medical history will be taken, a physical examination will be given, and sleep-related questionnaires will be administered to determine if the patient is eligible for any type of diagnostic sleep study. Most sleep clinics will have some sleep technologists who work in the day to perform specific day-time sleep studies, such as Multiple Sleep Latency Tests (MSLT), which will be discussed later in this chapter. The sleep clinic also is where patients are generally managed.

The sleep laboratory operates during the night hours. This is where the overnight sleep studies, or polysomnography (PSG) studies, take place. PSG studies are considered the "gold standard" for diagnosing various sleep disorders, particularly sleep apnea. Sleep laboratories also play a crucial role in research, helping to develop new diagnostic methods and treatments. Research conducted in sleep laboratories contribute significantly to the understanding of sleep and its impact on overall health and well-being. One of the most well-known sleep research laboratories in the world is The Stanford Center for Human Sleep Research. It was founded by Bill Dement, also known as "The Father of Sleep Medicine," and it became the world's first sleep clinic and laboratory in 1972. Now that we know what comprises a sleep center and what separates the sleep clinic from the sleep lab, we can start to understand more of its functions.

## Patient Flow / Navigation: Suspected OSA

Sleep Center & Lab

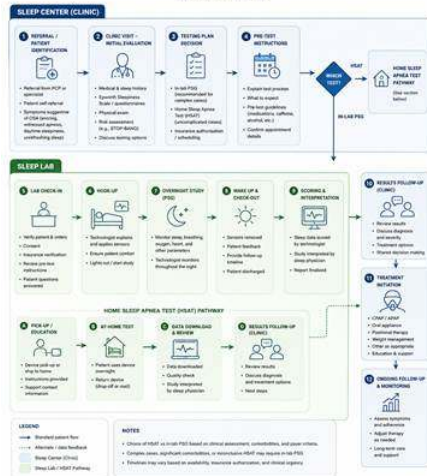


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## Functions of The Sleep Center

### The Sleep Clinic:

While the sleep clinic is focused on diagnosing and treating sleep disorders, there are several functions of the sleep clinic team that direct patient flow. Patients referred to the sleep center have their first care experience with the office staff. Sleep office staff usually includes front desk agents, scheduling associates, billing and coding personnel, and other persons who provide administrative assistance to the center. These staff members function as the first point of contact to the patient, making sure that whatever is needed before the patient comes in for their clinic visit is in order. More information about this team will be explored in chapter 2 ([Roles and responsibilities of Sleep Center Management](#)).

Patient management includes not only scheduling and testing but also the management of durable medical equipment (DME), such as ordering the correct prescribed Positive Airway Pressure (PAP) equipment and supplies for each patient, monitoring patient usage of prescribed sleep equipment, as well as ordering supplies needed to run the sleep lab. The management of patient DME is very important because insurance companies have specific requirements for patient usage in exchange for payment of equipment.

Unlike general sleep studies, which are typically conducted overnight to monitor patients while they sleep, daytime sleep clinics have an added function for individuals who may have problems related to daytime sleepiness, irregular sleep patterns, or disorders that manifest during waking hours. Their primary goal is to assess and manage conditions like narcolepsy, insomnia, sleep apnea, and circadian rhythm disorders that affect a person's ability to stay awake and function during the day.

One of the key functions of a daytime sleep technologist is conducting the **Multiple Sleep Latency Test (MSLT)**. This test measures how quickly a person falls asleep during the day, which helps identify conditions like narcolepsy or excessive daytime sleepiness. Another less common test is the **Maintenance of Wakefulness Test (MWT)**, which evaluates a person's ability to stay awake over a period of time. Both tests provide important insights about how a person's sleep patterns affect daily functioning. Home Sleep Apnea Testing (HSAT) is a simplified version of the PSG, and it is conducted by the patient at his or her home. HSAT is used primarily for diagnosing obstructive sleep apnea in adults and is suitable for patients who do not have significant comorbid conditions. Day technologists also are tasked with HSAT duties from start to finish. This requires reading orders, scheduling, setting up HSAT equipment, teaching the patient how to use the equipment, receiving and uploading tests, and scoring.

In addition to diagnostics, daytime sleep clinics can offer comprehensive treatment plans. These may include lifestyle modifications, cognitive behavioral therapy for insomnia (CBT-I), medications, or acclimatization clinics for CPAP machines—called PAP-Naps—for those who struggle with PAP use. The clinic often provides ongoing monitoring and adjustments to treatment, ensuring that patients can manage their conditions effectively and improve their quality of life.

Sleep clinics educate patients, caregivers, and communities on good sleep hygiene and help them understand the root causes of sleep issues. By addressing both the physiological and behavioral aspects of sleep disorders, these clinics play a critical role in improving daytime alertness and overall well-being.

### The Sleep Lab:

A sleep laboratory, or sleep lab, plays a central role in the clinical assessment of sleep disorders. The primary functions of a sleep lab are to provide excellent patient care while performing duties including diagnosis of sleep disorders, treatment intervention, patient monitoring, clinical research, equipment maintenance, troubleshooting, as well as various administrative duties. The sleep lab provides a controlled environment for conducting sleep studies and patient monitoring during the night.

The clinically recognized name for a night sleep study is a Polysomnography, or NPSG. A NPSG is a comprehensive recording of biophysiological changes that occur during sleep and is performed overnight in a sleep lab. The PSG system monitors various parameters such as brain waves (EEG), eye movements (EOG), muscle activity (EMG), heart rhythm (ECG), oxygen levels, airflow, and respiratory effort. All of these measures paint a picture of what is happening to the patient during sleep that may be causing different symptoms. During a diagnostic NPSG, a person may have drops in oxygen or continuous yet temporary collapses of the upper airway that may require the initiation of PAP.

Based on the results of a sleep study, physicians can develop individualized treatment plans, which may include lifestyle changes, medications, therapeutic Positive Airway Pressure (CPAP), supplemental oxygen, surgical interventions and referral to various subspecialty physicians.

Sleep labs involved in clinical research contribute to the understanding of sleep physiology, the impact of sleep disorders on health, and the development of new therapies.

### Standards of Accreditation

The most credible sleep centers have passed a standard of accreditation which ensures that sleep centers and laboratories maintain high standards of care. Accreditation means that an outside organization has come to evaluate the practices of the sleep center, including the clinic and the lab, to make sure that there is conformity to the standards set by the American Academy of Sleep Medicine (AASM). The AASM sets the standards for accreditation, focusing on quality assurance, patient safety, and continuous improvement. Quality assurance programs at sleep centers must include regular staff training, equipment maintenance, and standardized protocols for conducting sleep studies. Accredited sleep centers are more likely to provide accurate diagnoses, effective treatments, and better patient outcomes for the patients they serve. Sleep centers can be denied accreditation if they fail to meet the standards; however, they can fix any issues noted and reapply ([AASM](#)).

### Sleep Lab Environment and Patient Experience.

The patient experience in a sleep center is crucial for accurate diagnosis and patient satisfaction. Most sleep labs are designed to resemble a comfortable bedroom with a private bathroom, minimizing anxiety and ensuring a more natural sleep experience. There are several steps that go into creating a sleep lab environment that renders better overall patient outcomes.

Pre-Sleep Study Preparation begins before the patient arrives for their appointment. Patients are generally given instructions on how to prepare for a sleep study, such as avoiding caffeine or alcohol, following a regular sleep schedule, and bringing personal items to make the stay more comfortable. The technologist is critical in making sure that the physical environment sets the stage for the most successful overnight stay possible. This includes ensuring that the patient room is clean and free of any unnecessary clutter, offensive odors, checking room temperature, and making sure that all the necessary equipment is in good working order to minimize the interruption of the hookup experience. Ensuring that the patient is properly educated on what is being applied to his or her body and the function of the equipment is another great way to reduce patient anxiety and empowers the patient to make informed decisions during the study. Upon arrival, most patients will have to complete paperwork that can be overwhelming, depending on their healthcare literacy, eyesight, or other factors. This may cause anxiety for a patient that a technologist may not anticipate. Explaining the paperwork, reading it, and assisting a patient in filling out forms can be of tremendous help.

During an overnight study, sleep technologists attach sensors to the patient to monitor physiological parameters. The technologist remains in a control room to observe the patient throughout the night and can intervene if needed. Patients often ask if they will be watched over the camera during the night and may feel uncomfortable about this. Ensuring them that the camera is only there for their safety and for observation of abnormal activities – such as sleepwalking, sleep talking, movements, and position changes – helps patients feel more secure and less like they are on public display during a vulnerable experience. Speaking softly, maintaining a clean appearance, and using language that is appropriate and specific to the procedure helps to present to the patient that the technologist is knowledgeable, cares about their job, and cares about them.

After the sleep study, the data is analyzed by a technologist, interpreted by a sleep physician, and a follow-up appointment is scheduled with the patient to discuss the results and plan the next steps in management. At the follow-up appointment, the patient



also is given a chance to voice an opinion about the experience with the sleep lab and the technologist. Often, the technologist who served the patient has a greater effect than the physical environment and the wires. Since the sleep study experience can be very cumbersome to a person, is critical that sleep technologists create an atmosphere for the patient that is as enjoyable as possible. Focusing on the good things and showing empathy for a person's discomfort is a great skill that is developed in each technologist as they gain experience.

### Sleep Research and Clinical Practice

The field of sleep medicine is rapidly evolving with advancements in technology and shifts in healthcare delivery models. Emerging technologies, such as wearable devices and portable home monitoring systems, are making sleep diagnostics more accessible and less invasive. The rise of telehealth allows for remote consultations, follow-ups, and monitoring, expanding access to sleep care for patients in remote or underserved areas. Research is what helps shape clinical practice and sometimes clinical findings help create hypotheses for much-needed research.

Sleep medicine is increasingly integrated with cardiology, psychiatry, and endocrinology, given the interplay between sleep and overall health. This interdisciplinary approach enhances patient care. Sleep centers and laboratories have played a huge part in the advancements in diagnostic technologies, how people are treated, and the understanding of sleep disorders. Dr. Nathaniel Kleitman, the world's first researcher to focus completely on the research of sleep, established the first sleep laboratory at the University of Chicago around 1925. He designed and completed this project with the help of his students. Eugene Aserinsky and Dr. Kleitman were the first to discover rapid eye movements (REMs) during sleep and to correlate them to a sleep stage and dreaming. Since then, sleep labs have been the center point of developing, testing, and reporting on sleep and its effects on other health diseases, and vice versa. The types of research being conducted include clinical sleep research, behavioral, population-based epidemiological, neuroimaging, genetic, developmental sleep, environmental, and research cognitive function ([Shook](#)).

### Summary

In this chapter, we explored the foundational elements of sleep centers and sleep laboratories, their purpose, and their critical role in addressing sleep disorders. We began by defining the components of a sleep center, distinguishing between the daytime clinic and nighttime laboratory, and highlighting their complementary functions. The sleep clinic manages patient evaluations, diagnoses, and treatment plans while handling durable medical equipment (DME) and offering therapies such as Cognitive Behavioral Therapy for Insomnia (CBT-I). It also conducts daytime diagnostic tests like the Multiple Sleep Latency Test (MSLT) and Home Sleep Apnea Testing (HSAT). The sleep laboratory, operating at night, focuses on performing overnight polysomnography (PSG), a comprehensive diagnostic test for conditions like obstructive sleep apnea (OSA) and other sleep disorders. We discussed the importance of standards of accreditation, which ensure that sleep centers meet high-quality standards set by the American Academy of Sleep Medicine (AASM), leading to better patient outcomes. The chapter also addressed the role of the sleep environment and patient experience, emphasizing the significance of creating a comfortable, home-like atmosphere and fostering trust through empathetic interactions by sleep technologists.

Finally, we explored the broader contributions of sleep labs to research and clinical practice, including advancements in sleep diagnostics, treatment modalities, and the interdisciplinary integration of sleep medicine with other healthcare specialties. This chapter laid the groundwork for understanding how sleep centers operate and their vital role in improving patient health and advancing the field of sleep medicine. In chapter 2 we will begin to define each role in the sleep center, the scope of practice of each, and the impact each has on the sleep center patient.

#### 1.1: Chapter 1 References

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## CHAPTER OVERVIEW

### Chapter 2: Roles and Responsibilities of Sleep Center Management

This chapter will cover the roles and responsibilities of various management positions within the sleep lab.

#### Chapter Objectives

Differentiate roles, including their responsibilities

Identify social responsibilities

Define scope of practice for roles

### I. Medical Directors

A Medical Director (MD) is a professionally licensed physician or doctor (M.D., DO, PhD) whose roll is to oversee both the medical and operational aspects of the entire sleep center. Each sleep center has an MD who may or may not serve as a sleep physician. Although the MD must be a physician, he or she may have the option to only serve as the director and not as a practicing sleep physician. As a result, we will separate the role of MD from that of a sleep physician. The quality of institutional healthcare is strongly influenced by the effectiveness or the shortcomings of the MD. It is important that a MD shares the values of institutional policies, protocols, and procedures to ensure good success in delivering quality healthcare to diverse populations. Topics of management such as professionalism, fairness, medical records management, quality of care, patient satisfaction, medical education, and malpractice are discussed through the lens of institutional growth and improvement strategies; all fall within the MD's scope of responsibility.

Given that MDs collaborate closely with both sleep center staff and the institutions employing them, they must exhibit exceptional personal and professional qualities. These include ethics, empathy, medical expertise, fairness, professionalism, trustworthiness, approachability, and respectfulness. The American Academy of Sleep Medicine (AASM) suggests that an accredited sleep center will employ an MD who has the ability to ensure that all professional and technical staff are properly trained and certified in accordance with AASM standards and applicable national, state, and local regulations; oversees the daily testing operations and functionality of the facility; conducts quarterly reviews, updates, and adjustments to the quality assurance program as needed; provides educational sessions on sleep medicine topics or equipment for sleep technicians and other healthcare professionals when needed; and remains up to date on sleep medicine trends including products, procedures, technology, and more.

Patient care is the focus of every person employed at the sleep center; however, it is the duty of the MD to make sure that patient care audits are performed to create measures and implement improvements that lower risks and increase quality (Kossaify, A., Rasputin, B., & Lahoud, J. C, 2013). MDs may not always have direct patient contact; however, it is important that they are able to read patient charts, questionnaires, technologist notes, and other communications in order to understand how to adjust patient experiences.

#### Try it out:

You are the proud medical director of the Moraine Valley Sleep Center. During your bi-weekly patient quality audit, you came across two negative pre-study surveys for the same technologist on the same night. You are puzzled because this technologist has never had a negative pre-study survey. What would you do about this situation?

1. Say nothing and put it in the technologist's personal file.
2. Ask to speak with the technologist about it.
3. Write the technologist up.
4. Reach out to the patient to get more information.

### II. Physician Sleep Specialist

A sleep specialist is a physician who has gone through the specialty training in practicing sleep medicine and can be an M.D or D.O. The AASM, American Academy of Dental Sleep Medicine (AADSM), and the American Association of Sleep Technologists (AAST) are the three organizations that provide oversight and training for physicians who desire to achieve a specialty in sleep

medicine. In addition to the physician's medical training, each completes sleep-specific training and must pass the board examination for credentialing. A physician who has earned the sleep specialist credential may practice primarily or solely in sleep medicine if they desire. The most common physician specialists to receive training in sleep medicine are pulmonologists, neurologists, psychiatrists, cardiologists, and dentists. While primary care physicians can be a part of the multidisciplinary team for their patients, they are reluctant to provide interpretation and other sleep-specific needs because they do not specialize or have not been fully trained in sleep medicine patient care management (Hayes, Murray, Castriotta, Landrigan, & Malhotra, 2012).

The primary role of the sleep physician is to screen the patient to see if they may have a sleep disorder. They order testing, interpret diagnostic and therapeutic sleep studies, diagnose, or rule out sleep disorders. They prescribe therapeutic interventions such as PAP therapy, supplemental oxygen, and Cognitive Behavioral Therapy for Insomnia (CBTI.) Sleep specialists work with the patient's other care team members and medical equipment providers. If a referral is needed to another specialist because of an overlapping condition, they can provide it. Sleep physicians often provide education not only to the patient but to clinical members, other disciplinary teams, and the community at large.

### III. Advanced Practice Practitioners: Nurse Practitioners & Physician Assistants

Advanced Practice Practitioners (APP) are either Nurse Practitioners (ANP or NP) or Physician Assistants (PA) who work alongside physicians to assist in delivering patient care to the sleep patient. While an APP is not required to have a sleep specialty, they are required to have active licensure through a certifying board; some states also may require a state license to practice ( Zhao, Quadros, Nagraj, Wong, English, & Leckcivillize, 2024). Some of the positive contributions of having a sleep center that has an APP are reduced waiting times for patients, faster processing of records, better quality of care leading to greater satisfaction and patient outcomes, and better organizational flow (Halter, Wheeler, Pelone, Gage, de Lusignan, Parle, & Drennan, 2018).

In the state of Illinois, an ANP must work under a collaborative practice agreement, except when practicing within a hospital or ambulatory surgical treatment center; within that agreement they must collaborate and consult with their partnering physician at least once a month. The state of Illinois requires PAs to have an Illinois State Medical License conferred by the Board Physician Assistants of the state of Illinois. According to the policy of the American Medical Association (AMA), PAs are authorized to provide patient care services only so long as the PA is working under the direction and supervision of a physician or group of physicians (AMA Policy H-35.989, Physician Assistants).

The APP is often the first provider interaction a patient has within a sleep center. Their primary role is to initiate the patient's medical and social history, conduct a physical exam, order laboratory testing, and perform patient compliance follow ups. They also are responsible for ensuring appropriate documentation is provided, appointment and procedure coding is complete, and detailed notes on diagnosis, treatment plans, and patient concerns are entered into the patient's electronic medical record. They should also be consulting with the sleep physician who is supervising them on matters such as prescriptions, referrals, and any concerns they may have about a patient.

### IV. Managers & Supervisors

The manager's role is multifaceted and diverse relative to work environment. This role must be flexible to adapt to the ever-changing healthcare atmosphere. The sleep center manager is responsible for providing leadership and guidance to both the sleep practice and the healthcare workers who deliver services within the sleep center.

The sleep center manager must develop a process for the operational functions specific to the sleep center, which may vary depending on which area of concentration is needed in the unit. For example, a sleep supervisor would manage the technologist and his or her needs, such as supplies and work schedule. The sleep center manager would oversee the supervisor and various areas of overall sleep center performance, such as patient satisfaction, spending, and quality improvement.

Management and supervisors must meet often to organize and discuss needs, staff issues and solutions, decisions, and direction. Within hospital-based sleep centers, these meetings are often called "Huddles," where managers and/or supervisors meet with technologists to discuss important information, give updates, and check with techs on their nightly needs. Coordinating also is a very important responsibility of the manager and supervisor. Being able to build strong teams that are in alignment in the pursuit of a common goal is no easy feat. Being able to comprehend how a sleep lab is performing and making documented adjustments greatly increases the likelihood of success in reaching the sleep manager's goals and achieving the organization's mission. This leads to professional fulfillment as well. Acquiring the skill to firmly establish goals, communicate clear standards, measure outcomes, and remediate based on assessments is the gold standard of an effective sleep center manager.

## V. Certified in Clinical Sleep Health educators (CCSH) & Patient Navigators

A Clinical Sleep Educator (CCSH) plays a vital role in educating patients, families, and the community about sleep disorders, healthy sleep habits, and effective treatment options. They provide guidance on optimizing therapy, improving adherence to prescribed treatments, and overcoming barriers to care to enhance patients' overall quality of life. This position collaborates closely with Sleep Disorders technical staff, the Pulmonary/Sleep Clinic, healthcare providers, and DME vendors. This position may report directly to the sleep center manager, as well as the APP, physician, and MD.

Many of the essential functions in the CCSH role can mimic that of an APP. But it is important that we understand the clear scope of practice between the two roles. A CCSH plays a key role in supporting patients undergoing PAP therapy and other sleep-related treatments. They are responsible for scheduling and performing equipment downloads, reviewing data with patients, and determining necessary adjustments to treatment based on clinical guidelines. To ensure seamless patient care, they process essential paperwork, communicate changes to DME providers, and document all updates in the medical record. Collaboration with sleep lab staff and physicians is critical, as they assist in processing equipment orders following sleep studies, reviewing sleep charts for details, preparing prescriptions for physician signatures, and coordinating with DME providers to ensure timely adjustments.

In addition to equipment management, the CSE supports patients by addressing therapy-related concerns, such as mask fit, comfort, and adherence to treatment. They carry and respond to the PAP pager during clinic hours, assist with morning PAP education sessions, and handle patient inquiries regarding treatment. They also play a role in securing insurance authorizations for DME providers when needed. As part of their commitment to patient education, they lead CPAP education meetings for new users, coordinate support group meetings, and participate in broader educational initiatives.

Beyond direct patient care, the Clinical Sleep Educator engages in community outreach and advocacy efforts. They give presentations on sleep health topics such as sleep disorders, drowsy driving, shift work, and jet lag. Additionally, they staff sleep-related displays at corporate and school functions to raise awareness about the importance of healthy sleep habits. They also assist Surgical Services by evaluating and following up with bariatric surgery patients and conducting perioperative sleep screenings to optimize patient outcomes. Through these diverse responsibilities, the CSE serves as a vital resource in improving sleep health and overall well-being for patients and the community.

There is a lot of interdisciplinary contact between the CCSH and other departments as well as a lot of external contact between medical suppliers, families, communities, and beyond. Generally, the minimum education requirement for this position requires a person to possess at least an associate degree in a health care-related field – preferably sleep technology or respiratory therapy, RPSGT, RRT, or Nursing certification – one year of experience, knowledge of sleep disorders, therapeutic intervention experience (PAP), polysomnography and PAP analyzing experience, good customer service skills, and more.

## VI. Sleep Technologist

The Technologist works under the general supervision of the MD and under the direct supervision of an assigned supervisor to provide comprehensive evaluation and treatment for patients with, or who are suspected of having, a sleep disorder. Their responsibilities include conducting both in-center and home-based sleep studies, implementing diagnostic and therapeutic interventions such as PAP or oxygen therapy, and providing direct patient education and support. In addition to performing the diagnostic and therapeutic duties, a sleep technologist may also oversee other sleep technologists, if they are assigned the role of a Lead or Senior Technologist. This role requires the person be registered in sleep technology (RPSGT), ensuring expertise in patient care, sleep study procedures, and adherence to professional standards while on duty.

One of the primary responsibilities of a sleep technologist is to prepare and set up patients for sleep studies. This involves reviewing medical histories, verifying orders, and assessing each patient's specific needs, including physical, mental, and emotional considerations. Upon the patient's arrival, the technologist verifies his or her identity, obtains necessary consent, explains the procedure, and selects and calibrates the appropriate equipment for testing. They apply electrodes and sensors according to established protocols and ensure the patient is comfortable with the testing process. If necessary, they provide initial guidance on PAP therapy, including interface fitting and desensitization techniques.

During the sleep study, the technologist follows established protocols for various tests, including the Multiple Sleep Latency Test (MSLT), Maintenance of Wakefulness Test (MWT), parasomnia studies, and PAP or oxygen titration. They conduct physiological calibrations, monitor real-time data to ensure accuracy, identify and correct artifacts, and document key observations such as sleep stages, clinical events, and changes in procedures. When appropriate, they implement interventions such as PAP therapy adjustments or supplemental oxygen to ensure patient safety. At the conclusion of the study, they verify data integrity, repeat

calibrations, and instruct the patient on completing necessary post-study documentation. Sleep technologists also are responsible for scoring sleep studies using established guidelines and generating accurate reports for physician interpretation.

Beyond sleep studies, sleep technologists play a crucial role in patient education and follow-up care. They assist patients in understanding their diagnosis, treatment options, and therapy adherence strategies. Oftentimes, the patient's experience with the sleep technologist can encourage patient compliance with any therapeutic treatment needed in the home setting (Herbst, Metzler, Lenoci, McCaslin, Inslicht, Marmar, & Neylan, 2010). For home sleep apnea testing (HSAT), technologists provide instruction on equipment use, ensure proper data collection, and adhere to American Academy of Sleep Medicine (AASM) scoring standards. They also assist with portable monitoring equipment, following cleaning and disinfection protocols to maintain patient safety. Additionally, they may provide guidance on PAP therapy compliance, helping patients address mask fit, comfort, and tolerance issues if needed.

A sleep technologist must comply with all scope of practice laws, regulations, and safety standards, including infection control procedures and HIPAA guidelines to protect patient privacy. They perform routine maintenance and inventory management of sleep study equipment, ensuring all devices function properly. In addition, they maintain current CPR or Basic Cardiac Life Support (BCLS) certifications. Strong written and verbal communication skills are essential, as they interact with patients, families, and healthcare providers. Effective teamwork, customer service, and the ability to work independently are key aspects of the role. Technologists also are responsible for reviewing and evaluating sleep studies, providing feedback to staff, and assisting in the overall management of sleep center operations.

To qualify for this position, sleep labs may have various requirements, ranging from more relaxed to very competitive. According to the American Association of Sleep Technologists, there are statutory requirements for California, Delaware, Louisiana, Maryland, New Jersey, New Mexico, New York, North Carolina, Oregon, and Tennessee which require state licensure for sleep technologists to be able to practice (website<sup>[1]</sup> listed below). Other requirements may stipulate technologists must have successfully completed an accredited educational program leading to a certificate or associate degree in sleep technology as well as hold an RPSGT or RRT license in addition to possessing a state license. Alternatively, completion of a state-licensed or nationally accredited sleep technology program, or equivalent experience with documented proficiency, is acceptable. Privately owned laboratories may hire sleep technologists with no experience and are willing to train people with as little as no experience. These positions often are listed as "Sleep Tech Trainees" and will offer considerably less pay than someone with the experience and training.

The role of a sleep technologist requires physical stamina, as the job often requires standing for a prolonged period of time, handling equipment, and assisting patients. However, night shifts often will require sitting for hours; therefore, stamina to stay awake during the night is very critical. Technologists should be able to lift and move equipment weighing up to 50 pounds and perform tasks that require fine motor skills, such as applying sensors and calibrating machines. Vision requirements include close and distance vision, color differentiation, peripheral vision, and depth perception.

The work environment presents exposure to chemical disinfectants, cleaning solutions, and infectious agents, including blood-borne pathogens. Technologists must adhere to strict safety protocols to minimize risks. While most of the technologist's work is conducted in a sleep center, some responsibilities may extend to home sleep testing setups and patient education outside of the facility.

Overall, a sleep technologist plays a vital role in diagnosing and managing sleep disorders, ensuring high-quality patient care, accurate data collection, and adherence to best practices in sleep medicine. Through technical expertise, patient support, and collaboration with healthcare teams, they contribute significantly to improving sleep health and overall well-being.

## VII. Environmental Services

Environmental Services (EVS) is quite possibly the most important position within the health care environment. While often overlooked, EVS workers are the grounding factor in the control of public health infection control. They are trained to follow core infection prevention and control practices for the delivery of safe healthcare in medical settings. The Center for Disease Control (CDC<sup>[2]</sup>) has practice documents for all health care settings including that of a sleep lab. By properly disinfecting surfaces, disposing of waste, and following hospital protocols, they help reduce the risk of infections and contribute to the overall well-being of patients, staff, and visitors. Their work is essential in creating a safe and comfortable environment that supports patient recovery and efficient hospital operations.

The primary responsibility of EVS staff is to clean and disinfect hospital spaces following strict guidelines to prevent the spread of infections. This includes sanitizing high-touch surfaces such as door handles, bed rails, medical equipment, and workstations. They



are also responsible for properly handling and disposing of biohazardous waste, including used medical supplies, bodily fluids, and contaminated materials in compliance with hospital policies and regulatory standards.

EVS staff must follow infection control procedures, especially in isolation rooms and operating areas where patients are more vulnerable to infections. They ensure that patient rooms are cleaned thoroughly between occupants, changing bed linens, disinfecting surfaces, and replenishing necessary supplies. In addition, they help maintain hospital restrooms, hallways, and common areas, ensuring that these spaces are regularly cleaned and well-stocked with essential hygiene products.

Aside from routine cleaning duties, EVS staff may also respond to emergency situations, such as spills, contamination incidents, or outbreaks of infectious diseases. Their ability to act quickly and efficiently in these situations is critical to maintaining hospital safety. They may also work closely with laboratory management, staff, and infection control teams, including technologists, to ensure that all cleaning protocols align with the sleep center standards and patient care needs.

Hospital Environmental Services workers must maintain strong attention to detail to ensure that all cleaning and disinfection tasks are completed thoroughly and according to hospital standards. They should be knowledgeable about the proper use of cleaning chemicals, personal protective equipment (PPE), and specialized sanitation techniques to reduce infection risks. Physical stamina also is essential, as the job requires long hours of standing, walking, bending, and lifting supplies or equipment.

In addition to technical skills, EVS staff must practice good communication and work well on a team. They frequently interact with patients, visitors, and healthcare professionals, so professionalism and a respectful attitude are important. Discretion and an understanding of patient privacy regulations, such as HIPAA (Health Insurance Portability and Accountability Act), are also crucial as EVS staff may enter patient rooms and sensitive hospital areas. Problem solving skills and the ability to work efficiently under pressure are valuable, particularly when handling urgent cleaning tasks or responding to emergency situations.

Most hospital EVS positions require a high school diploma or equivalent, though some entry-level positions may not have formal education requirements. On-the-job training is typically provided, covering hospital cleaning protocols, infection control procedures, and the proper handling of hazardous materials. Some hospitals may require or prefer candidates with certifications in EVS, housekeeping, or credentials in infection prevention (e.g., Certified Healthcare Environmental Services Technician (CHEST) or Certified Healthcare Environmental Services Professional (CHESP)). Exams for these credentials are given through the Association for the Healthcare Environment (AHE<sup>[3]</sup>).

Experience in custodial work, housekeeping, or a healthcare setting can be beneficial, but many hospitals are willing to train new employees. Employees also must stay updated on best practices for infection control, particularly in response to public health concerns such as COVID-19 or other infectious disease outbreaks. Some hospitals provide ongoing training to ensure EVS staff remain knowledgeable about evolving cleaning and safety standards.

Hospital Environmental Services staff play an essential role in maintaining a safe and hygienic healthcare environment. Their responsibilities go beyond basic cleaning, as they help prevent infections, ensure regulatory compliance, and support the overall function of the hospital. With strong attention to detail, physical endurance, and a commitment to safety, EVS staff contribute to patient well-being and the smooth operation of healthcare facilities. While formal education requirements may be minimal, proper training and certification can enhance job prospects and effectiveness in this critical healthcare role.

1. List and explain the roles of physicians within the Sleep Center.
2. What is the difference in scope of practice for advanced practitioners and CCSH?
3. Explain, in detail, the difference between the manager and the supervisor in the Sleep Center?
4. Give an example of the flow or navigation of a patient with suspected OSA in the Sleep Center and Lab.
5. Describe the role of the CNA or MA in the sleep clinic, including staff who report directly to them.
6. What is the role of EVS workers and what is the difference between their responsibilities and the technologist's responsibilities within the sleep lab?

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[1] Link to the American Association of Sleep Technologies State Support Directory which was reference in the chapter text. [www.aastweb.org]

[2] Link to the U.S. Centers for Disease Control and Prevention Public Health website which was mentioned in the chapter. [www.cdc.gov]

[3] Link to the Association for the Health Care Environment website for information about CHESP certification which was mentioned in the chapter. [[www.ahe.org](http://www.ahe.org)]

## 2.1: References Chapter 2

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## CHAPTER OVERVIEW

### Chapter 3: Understanding Organizational Movement

At the end of this chapter, students will be able to develop a strategic plan from subjective data, be able to create assessment and measures for patient outcomes, understand health insurance company models, use proper sleep study codes, and know how to identify organizational goals.

#### I. Strategic Planning

A strategic plan is a document that outlines an organization's goals, vision, and mission, along with a timeline, measured in years, for each goal. Strategic plan documents are beneficial in guiding any organization, including individual families, toward success. A strategic plan should also include a design for achieving each goal. What makes a strategic plan effective? Effective steps to developing a strategic plan include identifying the issues within an organization, asking the right questions, and implementing the best solutions. The need for a strategy or a plan would not exist if areas in need of improvement did not exist. As Ralph "Waldo" Emerson said, "Our strength grows out of our weakness." Discovering areas that need improvement and focusing on those areas until each goal is achieved is critical in sleep medicine, healthcare, and in life. How can strategic planning and identifying areas for improvement in the sleep lab be achieved? Through assessment, measurement, implementation, analysis, and repetition of these practices.

#### II. Assessments, Measurement, And Outcomes

This section will focus on subjective data collected; this is the typical form of information gathered from employees and patients within the sleep center. Assessment is the organized and methodological collection, examination, and use of data. During a single shift, sleep technologists can ask patients to complete Epworth Sleepiness Scales (ESS), sleep history questionnaires (SHQ), patient satisfaction surveys (PSS), and more. These are all forms of subjective patient assessments, meaning that responses are based on how well or poorly patients believe they sleeps, or how well or poorly they have been told they sleep. It is not an objective measurement or history. Once the sleep study has been completed, the data collected from the polysomnography will transmit the objective data, showing whether the patient snores, kicks, or any other measure of sleep quality, reflecting or contradicting patient suspicions. Employees may be asked to take surveys, participate in focus group conversations, or be interviewed by leadership for their subjective opinions on topics to help develop key strategies. Once assessments are gathered, they can be measured.

Measurement happens when all the subjective data collected can be assigned to numbered groups (e.g., the average number of patients with ESS higher than 10; the engagement rates on PSS; or the annual number of reported sleep quality improvement from SHQ follow-ups). All measurements must be securely defined by a qualified number. Metrics are the tools used to collect information which is turned into measurable data. Keeping metrics or surveys consistent in format and wording assists in creating measurement reliability. This means that data collected over time will remain consistent. Even changing the font or colors on a survey can skew the validity/accuracy of the assessment, yielding incorrect outcomes. Yielding correct outcomes is vital because they provide results used to strengthen areas requiring improvement. A targeted strategy can then be made to create a desired outcome that yields improvement.

#### Health Insurance & Reimbursement

Health insurance is an agreement written in the form of a contract between an insurance entity and either an individual person or employer. This legally binding agreement states that a person will receive health benefits from necessary providers as long as the individual or employer pays the fees outlined by the insurance company. If the fees are not paid to the insuring company, the company is not obligated to cover any medical benefits from the date of the last payment received. Fees include, but are not limited to, monthly premiums, deductibles, and co-insurance.

Insurance premiums are the amount the individual pays out of pocket — typically every pay period or monthly — to the insurance company. Deductibles are the amount one may have to pay to the healthcare provider prior to payment by insurance. Co-insurance is a percentage that each person pays after he or she has met the deductible. Deductibles and co-insurance are commonly seen with PPO (Preferred Provider Organization) insurance plans. These plans help with healthcare cost sharing and allow people to see healthcare providers whenever and wherever they would like. However, the cost is generally higher than a Health Maintenance Organization (HMO) plan. An HMO plan has the same insurance premium standards as a PPO, but it usually does not come with

deductibles or co-insurance. HMOs do have co-payment fees, or a predetermined fee to see a provider. HMO plans require patients to have an assigned primary care provider to perform all screenings and provide referrals to specialists, if needed.

Insurance companies can be private companies, or they can be government funded companies. Medicare, Medicaid, and the Veterans Administration are the three government owned health insurance providers. The Health Insurance Marketplace is a government funded marketplace where individuals who may not be insured, employed, or eligible for private or government insurance, can find affordable, and, at times, no cost, healthcare insurance coverage. Sleep studies and visits within sleep centers are often paid for by insurance companies. The cost of sleep procedures varies by center, city, and state, making it difficult for patients to decide where to make appointments.

In the context of the sleep lab, reimbursement refers to the amount of payment that the insurance company will pay the sleep center for a service. Reimbursement may affect the amount each patient may have to pay for his or her service. In recent years, the American Academy of Sleep Medicine has been advocating in Washington D.C. for better reimbursement and additional measures of sleep health reform. Each type of sleep service is billed with procedure codes and each insurance company decides how much of the cost it pays for each procedure.

### Procedure Coding

The International Classification of Diseases and related health problems (ICD) is responsible for the creation of coding procedures for diagnostic services. The current ICD-10 is the most updated version of classifications and descriptions of diagnoses. Diagnoses from the ICD are indications for diagnostic procedures, paired with Current Procedural Terminology (CPT) billing codes. CPT codes are assigned to each study and describe what was done or used during that study. G codes are used to bill for HSAT's (Home Sleep Apnea Tests) and are considerably cheaper for insurance companies. The lower cost drives insurance companies to push for more HSAT testing, even though these tests not as accurate as full polysomnography studies. It is important that sleep technologists understand billing codes to ensure insurance companies are properly billed. Table A-1 shows the CPT and G codes for sleep studies, provided by the AASM.

A-1 Sleep Services Billing Codes

| CPT Codes | Description                                                                                                                                                                                                            | 2022 Reimbursement | 2023 Reimbursement |
|-----------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------|--------------------|
| 95782     | Polysomnography; younger than 6 years, sleep staging with 4 or more additional parameters of sleep, attended by a technologist                                                                                         | \$967.59           | \$962.40           |
| 95783     | Polysomnography; younger than 6 years, sleep staging with 4 or more additional parameters of sleep, with initiation of continuous positive airway pressure therapy or bi-level ventilation, attended by a technologist | \$1,024.69         | \$1,019.67         |
| 95800     | Sleep study, unattended, simultaneous recording; heart rate, oxygen saturation, respiratory analysis (e.g., by airflow or peripheral arterial tone), and sleep time                                                    | \$164.03           | \$150.80           |
| 95801     | Sleep study, unattended, simultaneous recording; minimum heart rate, oxygen saturation, and respiratory analysis (e.g., by airflow or peripheral arterial tone)                                                        | \$ 92.74           | \$93.87            |
| 95803     | Actigraphy testing, recording, analysis, interpretation, and report (minimum of 72 hours to 14 consecutive days of recording)                                                                                          | \$ 149.84          | \$140.63           |
| 95805     | Multiple sleep latency or maintenance of wakefulness testing, recording, analysis and interpretation of physiological measurements of sleep during multiple trials to assess sleepiness                                | \$ 427.04          | \$423.25           |
| 95806     | Sleep study, unattended, simultaneous recording of heart rate, oxygen saturation, respiratory airflow, and respiratory effort (e.g., thoracoabdominal movement)                                                        | \$ 93.44           | \$92.85            |

| CPT Codes | Description                                                                                                                                                                                                           | 2022 Reimbursement | 2023 Reimbursement |
|-----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------|--------------------|
| 95807     | Sleep study, simultaneous recording of ventilation, respiratory effort, ECG or heart rate, and oxygen saturation, attended by a technologist                                                                          | \$ 387.94          | \$391.40           |
| 95808     | Polysomnography; any age, sleep staging with 1-3 additional parameters of sleep, attended by a technologist                                                                                                           | \$ 685.55          | \$556.09           |
| 95810     | Polysomnography; age 6 years or older, sleep staging with 4 or more additional parameters of sleep, attended by a technologist                                                                                        | \$ 621.87          | \$615.05           |
| 95811     | Polysomnography; age 6 years or older, sleep staging with 4 or more additional parameters of sleep, with initiation of continuous positive airway pressure therapy or bilevel ventilation, attended by a technologist | \$ 649.21          | \$643.52           |

1. What is the purpose of a strategic plan?
2. Is the PSQ-III a form of measurement or is it a metric?
3. State the difference between managed care plans and list pros and cons for each.
4. What role does insurance play in cost control and what role do all sleep center employees play in cost control?
5. Is accessibility of care a form of an outcome or is it an assessment?
6. A 5-year-old patient is having an in-lab sleep study. The technologist applies EEG, ECH, leg, EMG, nasal pressure, thermistor, pulse oximetry, and respiratory effort belts. Four hours in the study, they apply CPAP to the patient. What is the proper billing code be for this study in 2022?

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## CHAPTER OVERVIEW

### Chapter 4: Legal and Ethical Issues in Sleep Medicine

At the end of this chapter, students will be able to explain sleep laboratory ethics, identify fraud or abuse of benefits, and create correspondence for patient rights and responsibilities.

#### I. Ethics in the Sleep Center

Sleep centers, as in any area of healthcare, face several different legal and ethical challenges related to patient care, privacy, professional standards, and more. Ethics describes the general standards of what is right (ethical) or wrong (unethical). In healthcare, each subset or specialty area has its own ethical code which is developed and agreed upon by practitioners. Ethical codes establish principles which guide moral standards in decision-making and patient care. Chapter 2 discussed the many different roles and responsibilities of those who work in sleep medicine. Each role has an associated licensing organization which sets an ethical code or standards of conduct for practitioners. It is important for managers to keep updated standards of ethics and conduct within the center so that, if any questions of unethical behavior arise, the ethical standards can guide problem solving efforts.

Ethically, sleep centers must prioritize patient safety, autonomy, and informed consent. Patients may undergo sleep studies, which can feel invasive and may require long-term use of medical equipment. Because of this, it is highly important that patients fully understand the procedures being performed and their purpose, the potential risks and benefits of having a sleep study or using therapeutic intervention, the importance of providing informed consent prior to treatment, and the potential costs associated. Conflicts of interest also can create ethical dilemmas. For example, patients may be prescribed CPAP for treatment. Medical equipment suppliers may offer prescribers or laboratories incentives to recommend only their products to patients. This could result in higher cost of treatment for patients and is a slippery slope leading to bribery, extortion and other legal problems. Transparency regarding treatment options is the best and most effective way to maintain ethical standards and minimize unnecessary risks.

Another ethical concern is the management of sleep disorders in vulnerable populations, such as children, the elderly, or individuals with mental health issues. Sleep centers must ensure that treatments are tailored to the specific needs of these groups and consider any potential risks or complications related to their conditions. For example, in pediatric sleep studies, parental and guardian involvement is vital in decision-making and ensures that procedures are as comfortable and non-invasive as possible. Sleep centers also must address disparities in access to care, ensuring that all patients, regardless of socio-economic background, have equal access to appropriate diagnosis and treatment options. Focusing on improvement is the best practice in avoiding legal issues.

Legally, sleep centers, just like other medical outpatient centers, must comply with the Health Insurance Portability and Accountability Act (HIPAA) of 1996. This act was built to help safeguard all patient data, including, but not limited to, medical records, questionnaires, and sleep studies. A breach of this confidential information could lead to significant legal repercussions, including fines and loss of licensure. As the use of technology increases in healthcare, the more important it is to be aware of violations, errors, and things that threaten the security of personal health information. Sleep centers must ensure that all treatments and interventions align with regulations and the practice's established medical standards. This includes, but is not limited to, ensuring all personnel have the proper licensures, providing secure electronic or paper medical record filing, and maintaining the use of FDA-approved equipment for diagnosis and therapy. In June 2021, Phillips Respironics, a prominent maker of PAP devices, released a recall on several of their devices after learning that a defect within their sleep therapy devices was leading to serious and life-threatening injuries. Many sleep centers used these products and, because the FDA's recommendation was to recall several of the devices, the management and directors of sleep labs were responsible for knowing, removing, and replacing any defective equipment. If defective equipment is found to be in use within a sleep lab and a patient develops symptoms following the use of defective equipment, the sleep center can be held legally liable for any damages the patient may have suffered.

#### II. Patient Rights and Responsibilities

Patient rights and responsibilities are central to the ethical and legal practice of sleep medicine. Patient rights ensure that individuals receive appropriate care and that practitioners respect patient autonomy and dignity in delivering this care. In medical practice, autonomy refers one's right to make decisions regarding his or her health care. One of the primary rights of patients in sleep medicine is the right to informed consent. Before undergoing any sleep studies, in the lab or at home, patients must be fully informed about the procedures, potential risks, benefits, and treatment alternatives. Informed consent also extends to the use of treatments like CPAP or other therapies, where patients must understand the nature of these interventions and their role in

improving their health outcomes. This empowers patients to make informed decisions about their care, fostering a sense of trust and collaboration between the healthcare provider and the patient. If any procedure or meeting between the patient and a provider of medical services within the sleep lab is done without proper explanation and consent of the patient, the sleep center could encounter legal issues. Patients have the right to access their medical records and seek clarification on any information related to their sleep disorders and treatments, which ensures transparency and open communication in their care.

In addition to these rights, patients have specific responsibilities in the context of sleep medicine as well. It is the responsibility of the patient and any caregiver to provide accurate and complete information to healthcare providers. This includes sharing detailed health histories, lifestyle habits, and symptoms related to sleep disorders. Patients' openness and honesty about their health and sleep patterns allows sleep specialists to make accurate diagnoses and recommend the most appropriate therapeutic treatments. Patients also are responsible for adhering to prescribed treatment plans, such as using CPAP machines or following specific sleep hygiene recommendations. Non-compliance or inconsistent use of recommended therapies can hinder the effectiveness of treatment, leading to suboptimal health outcomes. This is why it is important for patients to receive the best customer service and quality of care from every person they interact with in the sleep center. Studies have shown that patients are more likely to be compliant when their interaction with their sleep technologists were pleasant, encouraging, and informative (Gabryelska, Sochal, Wasik, Szczepanowski, & Białasiewicz, 2022).

It is very important for patients to actively participate in their own care as part of their patient responsibility. Active participation includes asking clarifying questions to understand any aspect of diagnoses or treatment plans. Sleep medicine, like other areas of healthcare, relies on a collaborative relationship between patients and providers. Patients should feel empowered to voice concerns, request alternative treatments, or ask for further explanation about their conditions. Open dialogue is essential, not only for effective treatment, but also for ensuring that patients feel respected and valued in their healthcare journey. By fulfilling both their rights and responsibilities, patients contribute to a more effective, ethical, and patient-centered approach to sleep medicine.

### III. Fraud & Abuse

Fraud and abuse in healthcare is a serious offense that can lead a person to hefty fines or even jail time. In sleep medicine, management must closely monitor ways fraud and abuse can manifest. There are various forms of fraud and abuse, potentially in areas that jeopardize patient safety, which increase healthcare costs and decrease the public's trust in the medical profession. One common form of fraud involves overbilling for services or equipment. For example, some sleep centers may bill for unnecessary or excessive sleep studies or treatments that are not warranted by the patient's symptoms. If a patient presents to the sleep center with complaints of insomnia, they should not be directly referred for a sleep study. The best course of action for that patient is to be first seen by a sleep psychologist who will gather subjective information and investigate why the patient believes they have insomnia. If further psychological therapeutic intervention and evaluation indicates a possibility of a sleep disordered breathing, then a home sleep study may be appropriate after evidence has been evaluated and a possibility of a breathing disorder during sleep has been established. Additionally, providers might bill for equipment, such as CPAP machines, that was either unused by the patient or prescribed without a proper diagnosis. These fraudulent practices not only harm the financial integrity of healthcare systems but also burden patients with unnecessary costs.

Earlier in this chapter, kickback arrangements between healthcare providers and medical suppliers were explained to be unethical; however, they are also a form of abuse. When healthcare providers receive compensation for referring patients to specific sleep centers or recommending certain treatments or equipment, conflicts of interest can arise. Conflicts of interest occur when providers prioritize financial incentives over patient well-being. For example, if sleep specialists receive kickbacks for referring patients to a particular company for CPAP machines, they may push patients toward this company regardless of whether it is the most appropriate option for their needs. Such practices undermine the ethical obligation to offer care that is in the best interest of the patient, potentially leading to suboptimal outcomes or unnecessary procedures.

Management must create a consistent monitoring system for fraudulent claims in the sleep center. Fraudulent claims can include the falsification of medical records or test results. In some cases, sleep centers may intentionally misrepresent the results of sleep studies to justify the prescription of expensive treatments or devices. This not only is unethical but also can be dangerous as patients may be treated for conditions they do not have, exposing them to unnecessary risks. For example, incorrect diagnoses of sleep apnea may result in patients being prescribed CPAP therapy, which could cause discomfort or side effects without addressing actual underlying conditions. Such actions are illegal and compromise the integrity of healthcare, making it essential for regulatory bodies to remain vigilant in detecting and addressing fraud and abuse within sleep medicine.

1. What types of ethical dilemmas might a sleep technologist come across in the sleep lab and what can the technologist do to correct it?
2. In your own words, define patient rights and responsibilities.
3. What is the difference between fraud and abuse?
4. Review the scenario below and write a brief description summarizing your employee's (Maria) actions, and describe what you should do to address the issue.

As a manager, you received a copy of the annual revenue report. You notice a total revenue increase of 25%. After a short celebration, as you do every year, you go through the billing files and noticed that the increase in revenue began one month after you hired Maria on the medical billing staff. You find that insurance companies have been charged twice for each patient for services each did not receive. You look at the current patient billing from the past three months and this corroborates your findings. What should you do?

#### [4.1: References Chapter 4](#)

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## CHAPTER OVERVIEW

### Chapter 5: Team Building and Communication

#### Chapter Objectives:

By the end of this chapter, students will be able to:

- Understand the principles of staff development and collaboration.
- Create motivational delegation strategies for improved teamwork.
- Cultivate awareness of their individual role and value within the team.

#### I. Co-workers as Teammates

In a sleep medicine setting, co-workers function as part of a highly interdependent care team. Each member – whether technologist, educator, physician, or administrator – contributes to the patient experience and the center's overall outcomes. Viewing co-workers as teammates rather than as individuals in isolated roles fosters cooperation and shared responsibility. The most effective teams operate with mutual respect, clear communication, and an understanding that collaboration improves both patient safety and staff well-being. According to the CDC and Healthy People 2030, improving workplace communication and team dynamics are essential in achieving better health outcomes and higher provider satisfaction (CDC, 2023; HealthyPeople.gov). Teams that actively support each other are more likely to reduce errors, address burnout, and create a culture of continuous learning. Daily huddles, open communication channels, and shared goals are examples of strategies that reinforce team identity.

Creating a team-based culture also means acknowledging contributions and recognizing when support is needed. Building trust within the team is a key factor in psychological safety, allowing individuals to express concerns and ideas freely. When sleep center staff consistently work as allies toward a shared mission of patient-centered care, it strengthens both internal morale and external patient satisfaction.

In professional sports, a franchise player is someone whose talent, experience, or leadership sets them apart. Think, for example about LeBron James in basketball, Tom Brady in American football, or Lionel Messi in soccer/football – these players consistently deliver high-level performances and are most often the glue for the team's success. However, even the franchise players do not win championships alone. They still depend on teammates, coaching staff, fitness trainers, support staff, and family members to execute the plays, strategies, and achieve the shared goal of winning.

The same principle applies in a sleep center. Some team members may have more experience or specialized knowledge that allows them to take the lead on complex cases, become a mentor to other staff, or be the team representative in professional settings. But they still are part of a larger care team. No matter how skilled one person is, the outcome of quality patient care depends on the collaboration of the team. Like a good sports team, a sleep center functions at its best when all players understand their roles, trust one another, and work toward the common goal of insuring a safe, effective, and compassionate care environment for each patient.

#### *Key strategies to build strong teams:*

- Practice active listening and inclusive communication.
- Give and receive feedback respectfully.
- Celebrate team wins and support others through challenges.

A culture of collaboration reduces workplace stress, prevents burnout, and improves patient satisfaction.

#### Think it Over

Think of a time when you were a part of a collaborative team. How did you feel during that time? Was there anything that could have been improved? If so, how would you have added to that improvement?

#### II. Stay in Your Lane

"Stay in your lane" is often misunderstood as a demand for decreased accessibility or a "paper ceiling," but in clinical practice to "stay in your lane" means honoring professional boundaries and clarifying roles. Sleep medicine involves contributions from many licensed and credentialed individuals, each with defined scopes of practice. Recognizing and respecting these boundaries helps to avoid replicating charges or services which can cause confusion and limits the concerns of liability.

For example, a sleep technologist may notice abnormal breathing patterns, heart arrhythmias, and brain activity on a patient during a sleep study. In fact, sleep technologists are trained to identify and calculate these abnormalities. The notes taken during the study are placed in the study and in medical records for interpretation, diagnosis, and billing. However, the technologist should not attempt to interpret or disclose the findings of the study to the patient or family members. Interpretation and patient diagnosis are not in the scope of practice for the technologist and to take these actions would be enormous liabilities for the technologist and the facility.

If the physician interpreting the study finds that the patient has a sleep disorder and the technologist told the patient that they did not have one, the technologist could be sued by both the patient and the sleep center for stepping outside of their “lane” or scope of practice. Likewise, a sleep health educator may help to guide therapy compliance, but should defer medical questions to a licensed provider. When roles are clear, each team member can operate confidently and efficiently, minimizing overlap and conflict.

Role clarity also contributes to job satisfaction.

Role conflict is a common cause of workplace stress in healthcare environments (OpenStax CC BY 4.0). Defining who is responsible for which aspect of care facilitates better delegation, reduces friction, and improves accountability. Staying in one’s lane is not about limiting involvement, but about building trust in one another’s expertise. Every role on the sleep team has a distinct responsibility. When individuals clearly understand what they are accountable for, it reduces role confusion and increases efficiency.

Tom Brady often is regarded as the NFL’s greatest quarterback in current history. His role as a quarterback is to handle throwing the football to other players who catch the ball and run the ball into the end zone to score a touchdown. He often speaks about how each teammate should know what his role is for the purpose of building trust and executing each play the team practiced. As in football, higher trust and accountability within a sleep care team and better patient outcomes from improved coordinated care are just a couple of ways being consistent in scope of practice helps to achieve goals.

When team members stay in their lanes, they also give others room to thrive in theirs. Knowing when to step up and when to defer to a colleague is a vital part of professional maturity. At times, this can be scary when in a new workplace, but it is important to make sure to continue to grow in knowledge and professional skills to ensure confidence as a technologist.

### III. Motivation, Edification, and Engagement

*Motivated* teams are built of motivated individuals. As a sleep technologist, where work often takes place overnight or with limited daytime management interaction, maintaining motivation requires intentional team development. Motivation begins with clarity of purpose. When staff understand how their work contributes to the broader mission, they are more engaged. Leaders and peers alike play a role in sustaining momentum. This can be done through feedback, recognition, and opportunities for growth. Recognizing small wins, offering mentoring, and investing in continuing education all contribute to a more engaged team. Providing snacks and meals also can go a long way to motivating the sleep technology team. A good leader cultivates motivation in tier staff by delegating tasks that match individual strengths.

*Edification*, or the act of uplifting others, especially is important in healthcare. Encouragement, peer support, and positive reinforcement improve staff morale and contribute to a healthier work environment. According to the CDC’s Workplace Health Promotion framework, employees who feel respected and supported are more productive and resilient (CDC, 2023). Staff members who actively build one another up contribute to lower turnover and better patient experiences. In a high-stakes environment like the ICU or Pediatric sleep medicine, emotional support among colleagues builds resilience against the complex patient situations one may encounter.

*Engagement* occurs when staff feel their work is meaningful and their voice matters. Including technologists and educators in problem-solving and decision-making fosters a sense of ownership. Offering opportunities for leadership, cross-training, and professional development also boosts morale and improves retention. A team that feels empowered is better prepared to handle the challenges of sleep healthcare. So, how do you know if your team is engaged? Team members who are encouraged will share ideas, take initiative, and participate in decisions. This shows that they feel comfortable and connected to their work and that they feel their opinion is valued.

#### *Tips for cultivating team motivation and engagement:*

- Delegate tasks in a way that develops confidence and new skills.
- Acknowledge both effort and outcome.
- Encourage open, judgment-free communication.

### Chapter Summary:

Team building and communication are not soft skills to be named on a resume; they are essential competencies for delivering safe and effective sleep care. By treating coworkers as teammates, respecting professional boundaries, and fostering motivation and mutual support, sleep center staff can develop a high functioning team culture that elevates both staff and patient outcomes.

### Chapter Discussion Question:

Below you will find the tips for team building and team motivation and engagement. Using these as a baseline, create two more tips for each topic. Write a one-page summary explaining your tips for each topic and why you think they will be effective.

#### Key strategies to build strong teams:

- Practice active listening and inclusive communication.
- Give and receive feedback respectfully.
- Celebrate team wins and support others through challenges.

#### Tips for cultivating team motivation and engagement:

- Delegate tasks in a way that develops confidence and new skills.
- Acknowledge both effort and outcomes.
- Encourage open, judgment-free communication.

#### Suggested Open and Public Domain Sources for Further Reading:

- Centers for Disease Control and Prevention (CDC). [Workplace Health Promotion](https://www.cdc.gov/workplace-health-promotion). [www.cdc.gov/workplace-health-promotion]
- [Healthy People 2030](https://odphp.health.gov/healthypeople)(opens in new window). Organizational and Community Interventions. [odphp.health.gov/healthypeople]
- Conerly, T. R., Holmes, K., & Lal Tamang, A. (2021). *Introduction to Sociology 3e: Groups and Organizations*. OpenStax. [openstax.org/books/introduction-sociology-3e/pages/6-introduction] (Licensed under CC BY 4.0)
- Logan, Paul [ImpulsiveClips]. (2025, May 20). [What Tom Brady Said To His Team During Historic 28-3 Super Bowl Comeback](https://youtu.be/YbIXEWmEQJI?si=qcrUba-yc5RVBmqj) [Video]. YouTube. [youtu.be/YbIXEWmEQJI?si=qcrUba-yc5RVBmqj]

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## CHAPTER OVERVIEW

### Chapter 6: Disparities in Sleep Medicine Access- Diversity, Equity, and Inclusion

#### Learning Objectives

- By the end of this chapter, students will be able to:
- Understand the concepts and contributing factors of healthcare disparities.
- Investigate trends in the usage of sleep medicine across different populations.
- Examine national and local initiatives aimed at reducing disparities in sleep medicine.
- Propose strategies to recruit and include diverse patient and technologist populations.
- Understand sociomedical demographics and how they impact sleep care.
- Identify common comorbidities and diseases that overlap with sleep-disordered breathing.

#### I. The Sleep Center Patient

As the population of the United States grows, the “typical patient” in the sleep center is becoming more diverse; yet disparities in access, diagnosis, and treatment continue. What is diversity? Diversity refers to the range of human differences including race, ethnicity, gender, age, socioeconomic status, religion, language, and physical ability. Historically, the “typical patient” in a sleep lab was considered an overweight person— mostly white males— who presented with complaints from wives of snoring. Occasionally, women were added into the mix if they too were overweight and complained of sleep issues.

Equity means providing fair access to resources and care, accounting for each patient's unique barriers. Inclusion involves ensuring that all individuals feel respected, represented, and supported within healthcare systems. In sleep medicine, the concept of diversity, equity, and inclusion (DEI) applies to how providers deliver care and communicate with patients from varying backgrounds. For example, studies have shown that Black and Hispanic patients are less likely to be diagnosed with or treated for obstructive sleep apnea (OSA) than their white counterparts, despite equal or greater risk.

These disparities may stem from reasons such as provider bias, lack of culturally competent education, or systemic barriers like poor insurance coverage. To address this, sleep centers have a responsibility to proactively tailor outreach and care practices to underserved communities, ensuring equitable access to diagnostics, follow-up, and therapy.

#### II. Socioeconomic Factors

Socioeconomic status (SES) involves a person's income, education, employment, and access to resources. Patients with lower SES often face significant barriers in accessing sleep medicine services. They may lack health insurance, transportation, or the flexibility to take time off work for sleep testing. Even when diagnosed, affording and maintaining PAP devices can be difficult. These economic constraints widen disparities in treatment and health outcomes.

Equity in this situation means acknowledging these limitations and responding with compassion and systemic solutions. Sleep centers can improve access by partnering with community health organizations, offering sliding-scale payment options, and supporting public health policies that expand coverage for diagnostic sleep services. Additionally, health professionals should be trained to recognize how poverty and resource scarcity impact compliance and patient engagement. DEI-informed care requires seeing beyond clinical symptoms to the social determinants affecting the patient's ability to succeed with treatment.

#### III. Cultural and Religious Influences

Cultural beliefs shape how individuals view health, illness, and treatment. Religion often plays a significant role in defining personal health choices and behaviors. In sleep medicine, cultural and religious norms may influence how patients perceive fatigue, snoring, or the use of medical equipment at home. Some cultures may view sleep disorders as a private or even shameful topic, which can prevent patients from seeking care.

Inclusion in healthcare involves adapting communication and treatment plans to align with a patient's cultural and spiritual values. For example, providing patient education materials in multiple languages or incorporating religious practices into scheduling (such as adjusting for prayer times or fasting periods) can build trust and compliance. Clinicians also must be careful not to impose their own cultural norms or assumptions onto people in their care. Technologists should be encouraged in practicing cultural humility, acknowledging what they don't know and being open to learning. These practices will help to enhance rapport with patients and patient outcomes.

#### IV. Age and Gender Identities

Sleep health concerns and access to care vary widely across different age groups and gender identities. Older adults are at high risk for conditions like sleep apnea and insomnia but are often misdiagnosed due to assumptions that poor sleep is a natural part of aging. Meanwhile, women frequently present with more subtle symptoms of OSA and may be dismissed or misdiagnosed with conditions like depression or anxiety.

Gender identity also plays a critical role in healthcare access. Transgender and nonbinary patients may avoid medical care due to past discrimination or fear of bias. Creating inclusive sleep centers requires not only awareness but also action, such as using gender-neutral language on forms, training staff in LGBTQI+ sensitivity, and ensuring that everyone feels respected during exams and consultations. RUSH University, UChicago Medicine, Northwestern University, and the University of Illinois are Chicago area medical providers with Gender Affirming Care Programs that provide patient navigation for people who wish to find comprehensive, supportive medical care. DEI in this context refers to removing both procedural and emotional barriers so that all patients can access and benefit from sleep health services.

#### V. Urban and Rural Care Gaps

Geographic disparities in healthcare access are well-documented, and sleep medicine is no exception. Urban areas typically have several options for people from which to choose, including hospital-based sleep centers and private sleep centers. However, barriers still exist within these options, such as language and cultural barriers, which can create a reduction of trust and lower compliance outcomes. Poor access to first-time or follow-up appointments can result in an increase in complications or hospitalization rates. Wait times for sleep specialist appointments at urban, hospital-based sleep labs may be as far in advance as four to six months from the initial request. Often, this does not include the appointment for sleep study, which can be another four to six month wait.

While urban areas may offer a range of sleep specialists and testing centers, patients in rural or remote regions often struggle with limited access. Long distances to clinics, lack of local providers, and poor broadband coverage for telehealth are significant challenges. Because of this, rural populations go undiagnosed or untreated for sleep disorders that significantly affect quality of life.

Equity in geographic access requires innovation and community engagement. Expanding the use of home sleep apnea testing (HSAT), remote CPAP monitoring, and mobile diagnostic units can help bridge these gaps. Training local providers and technologists from rural communities also helps safeguard cultural familiarity within the community as well as long-term sustainability of a sleep program. DEI in rural care means delivering high-quality sleep services regardless of ZIP code and recognizing that rural patients may face unique cultural and logistical challenges that require tailored approaches.

#### Think About It

What are some things that could increase access to comprehensive sleep studies for people living in rural or remote areas?

#### Chapter Summary:

Disparities in sleep medicine access are driven by a range of social, economic, cultural, and geographic factors. Diversity, equity, and inclusion are not optional concepts; they are foundational to providing effective, ethical, and comprehensive care. Understanding and addressing these disparities enables sleep professionals to ensure that every patient receives the care they need—without bias, without barriers, and with dignity.

#### End-of-Chapter Reflection Questions:

1. What are the most significant barriers to sleep care you see in your community?
2. How can sleep centers be more culturally and economically inclusive?
3. What strategies would you propose to improve technologist recruitment from underrepresented communities?
4. How do social and economic categories affect access to sleep medicine treatment?
5. Name three ways that a patient's religious or cultural views can both hinder and promote seeking help for issues of sleep?
6. Describe an instance where you observed or experienced a lack of cultural or religious sensitivity in healthcare delivery. What improvements could the healthcare provider have implemented in that situation?
7. Explain, in detail, barriers to sleep medicine for patients who live in both rural and urban areas.

### Suggested Open and Public Domain Sources for Further Reading:

Centers for Disease Control and Prevention (CDC). [Sleep and Health Disparities](http://www.cdc.gov/sleep). [www.cdc.gov/sleep]

National Institutes of Health (NIH) – [National Heart, Lung, and Blood Institute](http://www.nhlbi.nih.gov). [www.nhlbi.nih.gov]

MedlinePlus: [Sleep Disorders](http://medlineplus.gov/sleepdisorders.htm). U.S. National Library of Medicine. [medlineplus.gov/sleepdisorders.htm]

[Healthy People 2030](http://health.gov/healthypeople). Sleep Health Objectives. [health.gov/healthypeople]

[Office of Minority Health](http://minorityhealth.hhs.gov), U.S. Department of Health and Human Services. [minorityhealth.hhs.gov]

OpenStax: Health resources (licensed under CC BY 4.0). <https://health.gov/healthypeople> [openstx]



Health equality implies that everyone gets the same treatment, and health equity gives everyone a fair opportunity to obtain optimal health. (attribution: Copyright Rice University, OpenStax, under CC BY 4.0 license)

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## CHAPTER OVERVIEW

### Chapter 7: The Sleep Center Project

#### Welcome to Your Sleep Center Management Project!

During the course of the semester, you will be placed in groups to complete your final project. Each project will use the American Academy of Sleep Medicine and The Joint Commission's Standards and Guidelines for the accurate direction of your project. There will be five steps to this project, all with distinct due dates. Each project will be presented in class at the end of the semester for a final grade. Grading rubric, project templates, and submission instructions are available in Canvas Module 8.

#### Step 1. Choose your Project: Due the first week of February

1. Your team must research an innovative product to implement in the Moraine Valley Sleep Lab. The product can range from new PSG equipment to updated software, or anything that will help solve an issue for sleep technologists or the workflow of a sleep center.
2. Your team must design a sleep health equity campaign or public service announcement for the Moraine Valley Sleep Lab on any sleep topic that is affected by inequality. This may come in the form of educational materials, videos, or media press kits for advocacy of your topic.
3. Your team must produce a risk management plan for the Moraine Valley Sleep Lab. It should include a detailed plan including how to identify, assess, and respond to a potential risk of your choosing.
4. Your team must create a Quality Assurance plan for the Moraine Valley Sleep Lab. The plan will outline the processes and procedures to ensure that a product or service offered by the sleep lab meets quality standards.

#### Step 2. Report on your Project Design: Due the first week of March

Once your group has chosen your project, you will need to submit your proposed project design. Using the template in Canvas, include the following elements:

1. Topic/Title of project
2. Project Design (educational materials, PowerPoint presentation, video, podcast, etc.)
3. Provide role designations for each person, including their scope of work on the project

#### Step 3. Submit your Project Outline or Proposal: Due the last week of March

1. Upload your outline or typed proposal
2. Schedule a time for a private group discussion with instructor

#### Step 4. Submit Rough Draft: Due the last class before spring break

1. Turn in drafts and go over final editing once draft is returned

#### Step 5: Present Project

1. Project presentations will start during the third week of April

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## CHAPTER OVERVIEW

### Chapter 8: Sleep Technologist as a Leader

#### Chapter Objectives:

Identifying Leadership Styles

Taking Leadership Assessment

Implicit Bias Assessment

Who Am I as a Technologist?

Congratulations! You have learned more about the ins and outs of the sleep center. You know about the different roles on the sleep team, have discussed best practices and patient outcomes, gained an understanding of medical ethics and legal issues, and have insight into health system regulations. Now, let's talk about you – the sleep technologist – as a leader. We all are leaders in some way— whether leading ourselves in daily decisions, leading family or friends to the best places to grab lunch or leading the scoreboard in a favorite videogame, we all are leaders. Now it is time to ask yourself what type of leader you are.

Think about your experiences at work, school, or standing in line to get a state ID. What made those experiences good or bad? Was it your peers or your leaders? What would have made things better, not just for you, but for everyone who was having the same experience at that time?

You are going to assist in writing the final chapter of this text. We are going to dig into the type of leader you are and develop the outline of the type of leader you want to be. You are going to find a series of questions and prompts, all of which ask you to examine your skills, desires, biases, expectations, preferences, and motivations. Upon completing these questions, you will have enough information to help guide you into the best fit within your new career as a Polysomnography Technologist and as a leader.

#### Leadership Styles

There are many different styles of leadership, but within the context of a sleep technologist as a leader, we will briefly examine five of these styles, their characteristics, and a few pros and cons of each. The leadership styles we will examine include laissez-faire, autocratic, coaching, democratic and bureaucratic styles.

#### Laissez-faire leadership style

*Laissez-faire* is a French phrase that means “let it happen;” a similar American phrase is “let the chips fall where they may.” These phrases summarize the laissez-faire style of leadership. This style takes a mostly hands-off approach to leadership. It is one that allows members of the team to work on tasks completely on their own. People who work under this type of leader are expected to work independently in problem solving and make daily choices without interference, working within the lines of company policy and procedure for direction. This type of leader is willing to take accountability for the good and bad that happens within their team. This person has good discernment and trusts in a person's skills and abilities, which makes delegating tasks easy for them.

Pros and Cons of Laissez-faire leadership style

| Pros                                                                   | Cons                                                                         |
|------------------------------------------------------------------------|------------------------------------------------------------------------------|
| Encourages people to grow professionally and personally                | Lack of intervention can cause conflict                                      |
| Takes unnecessary pressure off employees and supports innovation       | Roles and duties can become confused                                         |
| Collaborative decision-making amongst team members                     | Employees may develop poor time management leading to retention issues       |
| Team members can be motivated by feeling trusted to work independently | People may feel disconnected and dissatisfied with secluded work environment |



### Autocratic leadership style

The focus of the autocratic leadership style is on clear separation between the leader and the worker. The workers here may have little input. Think of a basketball team: during a game, a player may give the coach some input based on his or her perspective playing the game; however, the play making decision comes from the coach, who is ultimately the person in charge. In this situation, if the player does not listen to the coach, they could get fired immediately – even if the team wins. If the coach never listens to the players' input and the result is either consistent loss or a big loss, then the coach may get fired. The workplace environment under this leadership style is typically very structured, organized, and oftentimes inflexible.

Autocratic leaders are often highly motivated and driven, self-confident, possess clarity in thought, reliability, and consistency.

Pros and Cons of Autocratic leadership style

| Pros                                                     | Cons                                       |
|----------------------------------------------------------|--------------------------------------------|
| Fast and well-organized decision making                  | Discourages team discussion                |
| Takes on stressful situations                            | Restrictions originality                   |
| Gives clear goals, oversight and direction               | Eliminates collaboration                   |
| Relieves employees of complex decision-making situations | Can limit personal and professional growth |

### Coaching leadership style

Being a part of a team is familiar to the coaching style of leadership. The coaching method is more personalized than other styles and values relationship building as a means of goal achievement. Making sure that employees have good work-life balance often is a key value to this leader. The coaching leadership style values improving employees through strengths training, or knowing their employees' strengths and weaknesses to empower them to build confidence in strong areas while producing gains in weak areas. Unlike the autocratic style, the coaching leader invites communication and works best by establishing relationships.

Coaching leaders are typically sympathetic, hopeful, supportive, forward thinking, and have the ability to discern the emotional needs of others.

Pros and Cons of Coaching leadership style

| Pros                       | Cons                            |
|----------------------------|---------------------------------|
| Develops close connections | Substantial time investment     |
| Open communication         | Reduce pace in goal achievement |
| Atmosphere of support      | Necessitates team compatibility |

### Bureaucratic leadership style

The bureaucratic leadership style is based on establishing a clear chain of command, rules, and processes of decision making. This leader works best when everyone has clearly defined roles and processes are in place for each.

Bureaucratic leaders are detail oriented, respected, disciplined, competitive, zealous, and often are misunderstood.

Pros and Cons of Bureaucratic leadership style

| Pros                             | Cons                                        |
|----------------------------------|---------------------------------------------|
| Clear understanding              | Discourages inventiveness                   |
| Removes confusion                | Uncompromising or not willing to change     |
| Impartial treatment of employees | Slow down of decision making due to process |
| Transparency                     | Reduced collaboration and communication     |

## Democratic leadership style

The democratic leadership style often is found in areas of government. This style gives emphasis to shared decision-making within the team. If a team member does not participate in the decision making, it is frowned upon by this type of leader. The team is uniquely organized to harness their unique individual talents or knowledge; therefore, if someone is not actively engaged, it could cost the team.

Democratic leaders are flexible, team focused, and good communicators with skills in persuasion and innovative thinking.

Pros and Cons of Democratic leadership style

| Pros                               | Cons                        |
|------------------------------------|-----------------------------|
| Team members have job satisfaction | Lengthy decision making     |
| Optimistic work atmosphere         | Can be indecisive           |
| Staff engagement                   | Inefficient processes       |
| Inclusivity                        | Negative outcome variations |

## Leadership Assessment

Now that we know a little more about these different leadership styles, it is time to take an assessment and to determine your closest leadership style. Below is a link to a free leadership assessment. Take this assessment and upload your results to the assignment on Canvas. Use the results of the assessment to help you answer the questions later in this chapter.

## Implicit Bias

If you were asked to talk about your last patient, what would you say? Would it be a technical answer, including how the hookup, study, and findings went? Would you be willing to confess what you really thought about your patient? If you did say how you really felt, would it expose your internal bias? Most people will not speak or act in a manner that reveals their true thoughts about a person. It is common to choose to try to understand ourselves better so that we may understand others. The best sleep technologists should know their beliefs and attitudes towards other people to ensure they provide the best service and care to all.

The Implicit Association Test (IAT) is an assessment that measures manners and beliefs that people have and reports these responses. Below is the link to the implicit bias test. Take the tests that are labeled, “Sexuality, Skin-Tone, Gender – Science, Age, and Disability.” After you have completed these assessments, write a journal entry of at least one page on your thoughts and perspectives about your results. Submit the journal entry on Canvas.

## Who Am I?

Now that you know more about the type of leader you are and your personal bias, let us examine your role as a sleep technologist and leader. Below, you will find prompt and a question. After reading the prompt, answer each question in at least one paragraph. Answer all questions on one document and submit your responses on Canvas.

### 1. What Kind of Technologist am I?

Reflect on your strengths, personality traits, and approach to patient care. How do these qualities shape your identity as a technologist? Consider how you handle challenges, communicate with patients, and contribute to your team. Think about your clinical experiences, remember the different personalities of fellow technologists, supervisors, and managers. Use those experiences to write about what you would like to model and what you want to avoid.

### 2. What Types of Environments Do I Work Best In?

Describe the work setting in which you feel most productive and fulfilled. Is it a fast-paced lab, a quiet clinic, or a collaborative teaching environment? What specific conditions— for example, team culture, structure, or autonomy— will help you thrive? What work environment do you believe is the worst for you? Is working independently in a four-bedroom lab uncomfortable for you? How would you express your concerns in uncomfortable work environments?

### 3. What Kind of Manager or Company Do I Work Best With/Within?

Think about the leadership style or workplace culture that brings out your best work. Do you value clear direction, independence, mentorship, innovation, or teamwork? What kind of support do you need to succeed? How do you think you can

bridge the gap between working nights and building relationships with management during the daytime? Is creating relationships with management important to you; why or why not?

**4. What Is Expected of Me?**

List the responsibilities and expectations placed on you as a technologist, from technical duties to professionalism and patient advocacy. How do these expectations align with your values and goals? If they do not, what steps can you take to make them align? From the perspective of a patient, what do you think should be expected of you as a technologist?

**5. How Can I Fulfill My Duties with Excellence?**

What does excellence mean to you in your role? Think about the skills, habits, mindset, and ethics needed to exceed expectations. What daily actions or long-term improvements can help you grow in your role? What is something you have found yourself to have a bias toward or against in your role? How do your personal beliefs affect the outcome of your nightly duties as a technologist, coworker, leader, and employee?

**6. How Can I Motivate Others?**

Reflect on how your attitude, actions, or communication style can inspire your peers, patients, or students. What strategies can you use to lead by example, build team morale, or encourage growth in others? Do you have influence? Describe what your influence looks like on the job.

**7. What Are My Five-Year Goals as a Technologist?**

Picture yourself five years from now. What do you hope to have accomplished professionally? Consider certifications, leadership roles, teaching opportunities, research, or work-life balance. How will you get there?

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